**Seminar case: Whose platform? The NHS Federated Data Platform**

CS5705 Ethics and Governance of Digital Systems · Lecture 4 seminar · 60 minutes

**Before the seminar:** Read this case and Carter, Laurie and Dixon-Woods (2015). Skim Powles and Hodson (2017) and NHS England's FDP frequently asked questions.

**Note:** this case is live and still changing. The facts below are correct as of mid-October 2026. If you find newer reporting, bring it to the seminar.

**The case**

**1. The problem**

NHS hospitals in England hold their data in hundreds of separate systems, for operating theatres, beds, waiting lists and discharges, that often cannot "talk to each other". Managers struggle to see, for example, which patients on a waiting list are ready for surgery, or which beds will free up tomorrow.

NHS England's answer is the **Federated Data Platform (FDP)**. It is software that sits on top of existing hospital systems and brings their data together in one place, so staff can plan operations, manage waiting lists and speed up discharges. NHS England says the FDP is vital for productivity and for bringing down waiting lists.

The word "federated" matters. Each NHS trust and integrated care board (ICB) runs its own copy, or "instance", of the platform and controls who can access it. Identifiable data is meant to stay within the local trust for direct care. NHS England runs a separate national instance, which uses de-identified data for planning.

**2. The contract**

In November 2023, NHS England awarded the contract to a consortium led by the US company **Palantir Technologies**, with Accenture, PwC, NECS and Carnall Farrar. It is worth up to **£330 million over seven years**: a firm three-year commitment, followed by options to extend. A separate contract went to IQVIA for "privacy enhancing technology" to de-identify data.

Palantir's NHS work began during the COVID-19 pandemic with a £1 contract in 2020. It was followed by about £60 million of contracts awarded without competition.

NHS England states that Palantir can use NHS data only to provide the service. Palantir cannot sell the data, use it to develop its own products or train AI models on it, and has no ownership rights in it.

**3. Why Palantir is controversial**

Palantir builds data platforms for governments, militaries and police forces, including work for US immigration enforcement. Critics argue that a company with this record, and with founders and leaders known for their political views, is the wrong partner to handle NHS patient data.

Transparency was an issue from the start. In 2024 the Good Law Project brought a legal challenge after NHS England published the contract with most of its pages blacked out. NHS England later

republished it with fewer redactions. Even so, about 100 pages, including those on how patient data is pseudonymised, were still withheld in 2026.

**4. What patients can and cannot do**

Patients **cannot opt out** of having their data processed by the FDP. In June 2026, a health minister told MPs that the national data opt-out "does not currently apply to products used in the NHS FDP", mainly because the data is used for direct care. Patients can still opt out of secondary uses such as research. Hospitals, by contrast, can choose not to use the platform.

In May 2026, NHS England confirmed that some Palantir staff had been given an "admin" role. This gave them access to a national data store containing identifiable patient data, to help build a new data collection platform. NHS England said access was security-cleared, approved by a director and regularly audited. Its own internal briefing had warned the change risked "loss of public confidence".

More than 47,000 patients have written to their hospital boards demanding withdrawal from the FDP. In September 2026, a health minister told MPs he believed distrust of Palantir lay behind a surge in NHS data opt-outs.

**5. Does it work?**

By June 2026, 168 of England's 214 NHS trusts had signed up, 123 were live, and all but one of the 42 ICBs had joined. The exception, Greater Manchester, uses its own in-house platform.

NHS England credits the FDP with more than 110,000 extra operations and a 15% reduction in discharge delays. In mid-2026, however, NHS England's methodology page added a caveat: it is not possible to "draw conclusions about cause and effect as other variables have not been controlled for."

The campaign group Foxglove used Freedom of Information requests to test the figures. It found that nearly a third of trusts using the platform performed fewer procedures after going live. It also found that one hospital accounted for 84% of a reported fall in outpatient waiting lists. The Office for Statistics Regulation is now assessing NHS England's claims.

**6. Professional and political pushback**

- **July 2025:** the British Medical Association voted that Palantir was "an unacceptable choice of partner". Palantir's UK head, Louis Mosley, told MPs that the BMA had "chosen ideology over patient interest".
- **February 2026:** the BMA said it would issue guidance advising doctors to limit their engagement with the FDP.
- **March 2026:** the Health Service Journal reported that nearly a third of trusts connected to the FDP in 2025 had not met NHS data security standards.
- **June 2026:** the Commons Science, Innovation and Technology Committee urged the government to use the contract's break point in early 2027 and to build an in-house or UK-based alternative. Its chair, Dame Chi Onwurah, said: "Vendor lock-in isn't inevitable, and the current position leaves us seriously exposed."

**7. The decision**

NHS England's firm commitment to the contract ends in early 2027. The government is reviewing the contract and is expected to decide by the end of 2026 whether to continue with Palantir. Its options include:

- **Continue** the contract, arguing that the platform is embedded and delivering benefits.
- **Exit** at the break point and move to an in-house or UK-based alternative, as Greater Manchester has done.
- **Continue with conditions**, such as stronger transparency, independent evaluation of benefits, and new limits on access.

**Seminar questions**

Answer all six questions. Use the concepts from Lecture 4, trust, explainability, privacy by design and social contract theory, and support your arguments with evidence from the case and the reading.

1. Identify **three** stakeholders affected by the Federated Data Platform. For each, explain one ethical concern the case raises for them.
2. NHS England credits the FDP with more than 110,000 extra operations but says it cannot show cause and effect. Using London's (2019) argument about **trust, accuracy and empirical testing**, evaluate whether these benefit claims are a sound basis for public trust.
3. Patients cannot opt out of the FDP, but hospitals can. Using **social contract theory**, and the idea of a social licence from Carter et al. (2015), evaluate whether the FDP keeps or breaks the social contract between the NHS and its patients.
4. Evaluate how far the FDP's design, local instances, privacy enhancing technology and contractual limits on Palantir, meets the principle of **data protection by design and by default**. Consider the decision to give some Palantir staff access to identifiable data.
5. Who should be **accountable** if the FDP harms patients or fails to deliver its benefits: NHS England, individual trusts, Palantir, or ministers? Evaluate how well current governance mechanisms (the contract, Parliament, regulators, professional bodies and the courts) hold them to account.
6. The government must decide whether to continue the contract. **Recommend** whether to continue, exit, or continue with conditions, and propose **three governance measures** to support your recommendation. Justify each using an ethical perspective and the reading.

**Reading**


- Carter, P., Laurie, G.T. and Dixon-Woods, M. (2015) The social licence for research: why care.data ran into trouble. *Journal of Medical Ethics*, 41(5), 404–409. Open access: PubMed Central
- Powles, J. and Hodson, H. (2017) Google DeepMind and healthcare in an age of algorithms. *Health and Technology*, 7(4), 351–367. Open access: PubMed Central

- NHS England, NHS Federated Data Platform: frequently asked questions. Read it critically: it is the platform owner's own account.
- Optional: Understanding Patient Data and Ada Lovelace Institute (2020) *Foundations of fairness: where next for NHS health data partnerships?*

**Case sources**

- Digital Health (November 2023): NHS England awards £330m FDP contract to Palantir
- Digital Health (February 2024): Good Law Project sues NHSE over heavily redacted FDP contract
- Pulse (9 July 2025): Palantir accuses BMA of choosing ideology over patient interest
- Socialist Health Association (16 February 2026): BMA to tell members to limit use
- The Register (12 May 2026): NHS England confirms Palantir staff can access patient data
- The Register (3 June 2026): UK lawmakers call on government to ditch Palantir NHS contract
- The Register (13 June 2026): NHS patients can't opt out of Palantir's data platform
- The Deep Dive (26 June 2026), reporting the FT on NHS England's caveat to its benefit claims
- The Register (8 July 2026): NHS told to show its working on Palantir platform benefits
- Resultsense (1 October 2026): Pressure grows to end Palantir's NHS data contract